



PO No :PO2921610454-600



Customer Name	: Mr.SRINIVAS	Collected Via	: TATA 1MG BANGALORE
Age/Gender	: 58/Male	Referred By	: Dr.
Lab Visit ID	: MGB1333850	Collection Date	: 04/Aug/2025 07:51AM
Barcode ID/Order ID	: D20588433 / 13763137	Report Date	: 04/Aug/2025 08:01PM
Sample Type	: Fluoride Plasma F	Report Status	: Partial Report

BIOCHEMISTRY

DIABETES SCREENING

Test Name	Result	Unit	Bio. Ref. Interval	Method
FBS (Fasting Blood Sugar)				
Glucose - Fasting	83	mg/dL	70-99	Hexokinase/G-6-PDH

Fasting Plasma Glucose (mg / dL)	2 hr plasma Glucose (mg / dL)	Diagnosis
99 or below	139 or below	Normal
100 to 125	140 to 199	Pre-Diabetes (IGT)
126 or above	200 or above	Diabetes

Reference : American Diabetes Association

Comment:

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes

Plasma Glucose Goals	For people with Diabetes
Before meal	70-130 mg/dL
2 Hours after meal	Less than 180 mg/dL
HbA1c	Less than 7%



This test has been performed at
TATA 1MG BANGALORE
 Address: No 607, Ground, 1st & 2nd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr Ashwin Kumar A.S
MBBS M.D (Biochemistry)
Consultant Biochemist
Reg No:68123





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Sample Type	: Serum	Report Status	: Partial Report

BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Calcium	7.7	mg/dL	8.4-10.2	Arsenazo III

Comment:

Increased in: Hyperparathyroidism primary and secondary, Acute and chronic renal failure, Following renal transplantation, Osteomalacia with malabsorption, Acute osteoporosis, Malignant tumours (specially of breast, lung and kidney), Drugs: Vit. D and A intoxication, Diuretics, estrogen, androgen, tamoxifen, lithium

Decreased in: Hypoparathyroidism, Surgical and Idiopathic, Pseudohypoparathyroidism, Chronic renal disease with uremia and phosphate retention, Malabsorption of Calcium and Vit.D, obstructive jaundice, Bone Disease (Osteomalacia and rickets), Drugs: Cancer chemotherapy drugs, calcitonin, loop-actives diuretics, Hypomagnesemia, Hypoalbuminemia

Creatinine with eGFR (18 Years & Above)

Creatinine	5.74	mg/dL	0.6-1.3	Kinetic Alkaline Picrate
Glomerular Filtration Rate (estimated)	11	mL/min/1.73m ²		Calculated

- Calculation of eGFR is based on 2021 CKD-EPI creatinine equation, recommended method for estimating GFR in adults (>=18yrs). Estimates GFR using serum creatinine, age and sex.
- GFR categories defined acc. to KDIGO 2024 Clinical Practice Guideline for the Evaluation and Management of CKD.
- eGFR_{creat} levels less than 60 ml/min/1.73 m² should be considered as "decreased GFR".
- It is advised to estimate GFR using Cystatin C for confirmation of CKD in patients with eGFR_{creat} value 45-59 ml/min/1.73 sq.m without evidence of kidney damage and in specific circumstances when eGFR_{creat} is less accurate e.g. race, muscle mass, exercise, pregnancy, dehydration and nutritional status.

Interpretation:

GFR categories in CKD

Category	GFR (ml/min/1.73 sq.m)	Terms
G1	≥ 90	Normal or high
G2	60-89	Mildly decreased
G3a	45-59	Mildly to moderately decreased
G3b	30-44	Moderately to severely decreased
G4	15-29	Severely decreased
G5	< 15	Kidney failure

Criteria for CKD (Either of the following present for > 3 months)

NABL certificate and scope



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Markers of kidney damage (one or more)	Albuminuria (ACR >30 mg/g) Urine sediment abnormalities Electrolyte and other abnormalities due to tubular disorders Abnormalities detected by histology Structural abnormalities detected by imaging History of kidney transplantation
Decreased GFR	GFR <60 ml/min/1.73 sq.m (GFR categories G3a-G5)

Source: KDIGO 2024 Clinical Practice Guideline.

Kindly correlate with clinical history and other relevant parameters

Critical Result-(Creatinine) . Please consult your doctor immediately.**Advice -Kindly correlate clinically.Repeat testing may be done on a fresh sample, if clinically indicated.**

Comment:

Creatinine Interpretation: Creatinine is a more precise and sensitive marker for renal disease than Blood Urea Nitrogen (BUN), especially for diagnosing renal insufficiency, adjusting dosages of drugs, and monitoring renal transplant recipients. Serum creatinine level can serve as an indirect indicator of reduced skeletal muscle mass. Serum creatinine measurement is used in estimating the Glomerular Filtration Rate (eGFR) for people with Chronic Kidney disease (CKD) and those with risk factors for CKD (Diabetes Mellitus, hypertension, cardiovascular disease, and family history of kidney disease); Present calculation is valid for ages 18 years and above.

- Increased in:** Blockage in the urinary tract, pre- and postrenal azotemia, impaired kidney function, Loss of body fluid (dehydration), Muscle diseases such as gigantism, acromegaly.
- Decreased in:** Pregnancy, certain drugs (e.g., cimetidine, trimethoprim), Myasthenia Gravis, Muscular dystrophy.

*** End Of Report ***

Pending Test-HbA1c (Glycosylated Hemoglobin), Hemoglobin

Conditions of Laboratory Testing & Reporting:

Test results released pertain to the sample, as received. Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the interpreting clinician. Result delays may happen because of unforeseen or uncontrollable circumstances. Test report may vary depending on the assay method used. Test results may show inter-laboratory variations. Test results are not valid for medico-legal purposes. Please mail your queries related to test results to Customer Care mail ID care@1mg.com

Disclaimer: Results relate only to the sample received. Test results marked "BOLD" indicate abnormal results i.e. higher or lower than normal. All lab test results are subject to clinical interpretation by a qualified medical professional. This report cannot be used for any medico-legal purposes. Partial reproduction of the test results is not permitted. Also, TATA 1mg Labs is not responsible for any misinterpretation or misuse of the information. The test reports alone may not be conclusive of the disease/condition, hence clinical correlation is necessary. Reports should be vetted by a qualified doctor only.

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